

Understanding University Students' Experiences on Multi-domain Help Seeking Platform 'Fruto': Vignettes Study

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Abstract

While various mental health services exist for college students, barriers such as lack of mental health knowledge and stigma prevent them from seeking needed help. A campus-oriented mobile platform developed to facilitate help-seeking among Korean college students was used in our study. This platform integrates features, including counseling scheduling, offline campus program sign-ups, self-screening tools, and mental health information. Our goal is to understand how Korean students use and perceive the multi-domain help seeking platform. Using the vignette method, we interviewed 16 students and analyzed the data through thematic analysis. The platform facilitated help-seeking by offering mental health information and connecting users to campus offline service. Our findings also highlighted the importance of everyday mental health-related content rather than overly emphasizing psychiatric terminology. The study concludes that designing digital mental health services requires reducing the stigma associated with seeking help and adopting an approach that is well-integrated with the campus environment.

CCS Concepts

• Human-centered computing → Empirical studies in HCI.

Keywords

Mental health, Korean college students, Help-seeking, Mobile platform

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1 Introduction

Mental health problems are common among college students and pose a significant threat. More than one-third of U.S. college students reported experiencing depression sufficiently severe to impair their functioning in the previous year, and approximately 10% indicated that they had seriously considered attempting suicide [3]. As college students transition from adolescence to adulthood and adapt to new roles and environments, they face various mental health challenges owing to social, financial, and academic pressures. Most lifelong mental disorders first emerge during college years [19]—often defined as emerging adulthood—and are regarded as major concerns that threaten both individual and societal development. In particular, 74.5% of South Korean college students reported experiencing anxiety stemming from pressures related to interpersonal relationships, employment, and career decisions [31], and 43.2% exhibited depressive symptoms [14], suggesting that the issue is more serious than in other developed countries.

Although mental health problems among Korean college students are severe, the proportion of students seeking help remains low. According to the Korean Ministry of Health and Welfare, only 22.2% of Koreans seek professional help, compared with 41.3% in the United States and 46.5% in Canada [21]. The primary reason for this low help-seeking rate among students is their insufficient recognition of the need for mental health services. Furthermore, several students do not actively seek help because they consider the stress experienced during college life to be normal or cite a lack of time [12]. Notably, Asian students, including those from Korea, often conceal their mental health issues owing to cultural norms

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that place a high value on saving face [27, 30, 39]. When concerns about negative judgments or prejudice from others are internalized, individuals develop self-stigma and begin to view themselves negatively [10]. Such self-stigma can exacerbate mental health problems and hinder help-seeking behaviors [40]. This study explored the experiences of Korean college students using mental health platform to identify requirements that mitigate stigma and barriers to help-seeking and to incorporate these findings into service improvements.

Mental health services-based digital technology helps college students seek assistance without experiencing an excessive psychological burden. College students who are comfortable with online environments can access services more freely through anonymity and convenience [7, 11]. Most studies have focused on quantitatively evaluating the effectiveness of interventions developed around specific features [22, 29]. The importance of research on university counseling centers—which play a key role in mental health interventions for college students—and integrated mobile platforms designed to enhance their functions is still being emphasized [24]. It is also important to obtain feedback from the general student population, not only from students with mental health challenges, after they have actually used the platform [42]. Their feedback will contribute significantly toward qualitatively improving the components of the platform and effectively applying it to campus environments.

In June 2024, “Fruto” was developed—a mobile platform that enables two-way communication in both online and offline environments with the content provider, the on-campus counseling center. Fruto integrates multiple functions—including mental health information provision, screening, counseling scheduling, and mental health program registration—into a single application designed for college students. To understand how students with diverse usage motivations experience Fruto, we conducted interviews using a virtual scenario (vignette) approach. The specific research questions were as follows:

RQ1. How do Korean college students with various usage motivations use and perceive a mobile mental health platform?

RQ2. What design principles should be considered for a mobile mental health platform to function effectively in real campus environments?

By identifying Korean college students’ requirements for the platform, this study aimed to contribute to the design of effective computing systems that promote mental health management and help-seeking among college students.

2 Related Work

2.1 Digital Mental Health Services for Promoting Help-Seeking

In the field of mental health, help-seeking refers to “the process of seeking external assistance to address mental health issues [33].” Young adults often refrain from seeking formal help because of misconceptions about their mental health and self-stigma [35]. Various interventions aimed at reducing the stigma and improving mental health literacy have been explored to encourage students to seek help. Previous studies have focused on providing mental health information to foster a correct understanding of mental disorders and

raise awareness about the need for help-seeking [28]. According to Talor-Rodgers et al. (2014) and Howard et al. (2018), online psychoeducation can increase help-seeking intentions among young adults and reduce stigma associated with mental illness [18, 38]. Wiljer et al. (2020) found that a mobile app that informs people about nearby mental health resources reminds them of the importance of asking for help [43]. Some studies have attempted to promote help-seeking through self-screening interventions. Lee et al. (2022) reported that incorporating personalized feedback alongside online self-screening tools improves college students’ understanding of mental health and their motivation to manage it [26]. Supporting help-seeking processes such as counseling through remote methods is also effective in reducing temporal and spatial barriers, as well as social stigma [15]. A longitudinal study by Baldofski et al. (2023) demonstrated that online chat counseling services not only enhance self-efficacy among young adults but also foster additional help-seeking behaviors [4]. Numerous researchers continue to investigate whether digital mental health services effectively promote help-seeking. However, existing studies are often limited because they tend to offer only a single feature of mental health support. In this study, we employ a multi-domain mobile platform that integrates multiple core features designed to facilitate help-seeking. This platform targets general students and allows them to access offline mental health services through an online interface. These core features are described in detail in section 3.2

2.2 Practical Implementation of Digital Mental Health Services in Campus Settings

Although research on digital mental health interventions has grown, a significant gap remains between theoretical findings and their practical application in real campus environments [23]. To effectively integrate digital mental health services into campus systems, merely studying the efficacy of services is insufficient. Additional research is required to determine how these services can be utilized in campus contexts. According to Lattie et al. (2022), digital mental health apps offer potential benefits in improving college students’ mental health. However, strategies to increase user engagement and participation remain necessary, and emphasis should be on integrating digital tools with existing offline mental health services on campus [24]. Min et al. (2024) confirmed that a mindfulness app positively influenced Korean college students’ mental health [29]. However, because this study did not focus primarily on user experience, further investigation is needed to understand how students experience a platform organically connected to the campus environment. In this study, we aimed to bridge this gap by exploring how Korean college students interact with a multi-domain mental health platform that integrates both online and offline campus mental health resources. By analyzing user experiences, we provide insights into how digital mental health platform can be effectively implemented in campus settings.

3 Method

3.1 Participants Recruitment

We recruited participants through Everytime, a mobile campus community platform that required student verification. All participants were undergraduate or graduate students from the same

university. Ultimately, 16 undergraduate and graduate students volunteered, and all participants received a payment of 20,000 KRW (approximately USD 13.6).

3.2 App Design and Rationales

This section describes the design and rationale of the five main features of the platform. First, we included a self-diagnosis function that enables users to assess their current mental health status using an online self-report screening tool. Specifically, the screening component included measures of six mental health issues: anxiety (Generalized Anxiety Disorder-7, GAD-7), depression (Patient Health Questionnaire-9, PHQ-9), alcohol use (Alcohol Use Disorders Identification Test-Korean version, AUDIT-K), adult ADHD (Adult ADHD Self-Report Scale Version 1.1, ASRS-V1.1), sleep quality (Pittsburgh Sleep Quality Index-Korean version, PSQI-K), and suicidal risk (P4 Suicidality Screener, P4). To support more effective management, we added a follow-up component that recommends counseling or relevant content based on each user's screening results, particularly for those identified as high-risk [16].

Second, a "Health News" feature providing information about various mental health topics such as depression, anxiety, and stress is present. This feature includes a wide range of text-based content, from recommended readings on mental health to columns written by campus psychiatry specialists. Because mental health literacy influences students' attitudes toward help-seeking [20], delivering accurate mental health information is crucial. Furthermore, as students often spend time in public or shared living spaces, we opted for text-based materials to allow private access to potentially sensitive information [25].

Third, we incorporated a counseling appointment feature for users who needed counseling, enabling them to schedule in-person sessions on their preferred dates and times. As college students are accustomed to online environments and prefer anonymity, they often favor remote methods when disclosing mental health concerns [15]. A study in Hong Kong reported that online counseling can effectively reduce psychological barriers to seeking help, yet integrating offline counseling is more effective in addressing practical issues [9]. Reflecting on these findings, the Fruto counseling appointment feature supports reservation of offline counseling sessions via a mobile interface.

Fourth, users can register for offline mental health programs, such as meditation sessions or group counseling. College students often use digital mental health tools to connect with resources in physical spaces—such as on-campus mental health events—instead of solely online [25]. Moreover, when mobile mental health platforms are integrated into existing campus programs, they are likely to exhibit higher acceptability and sustainability [8].

Finally, the "Center Introduction" feature provides information about the on-campus healthcare center, which operates counseling services and physical and mental health clinics. Using this feature, students can access the locations and contact details of the resources from which they receive assistance. Offering emergency contact information is a key function, as it can be beneficial in panic or suicidal crises [2]. Additionally, disclosing mental health resources can positively influence the perceived credibility of such services [1].

Screenshots of the mobile platform featuring these core functions are provided in the Appendix A.

3.3 Interview Procedure

To explore students' experiences with *Fruto*, we conducted semi-structured interviews using a vignette approach. A vignette is defined as "a short story about a hypothetical person in a specific situation, used in research by asking interviewees to respond to that situation" [13]. This approach allowed us to consider a wide range of user motivations, from students who could benefit from counseling to those who use the app simply out of curiosity. This method also helps minimize participants' psychological burden and encourage honest feedback, particularly for those hesitant to disclose their reasons for usage due to privacy concerns [5, 36]. In this study, we developed four potential user groups based on different usage motivations and constructed four scenarios that reflected each group's circumstances. Two scenarios were randomly assigned to each participant. The original versions of the scenarios provided to participants are included in the Appendix B.

<Summary of Vignettes for the Four Usage Motivations>

- Peer supporter (PS): Students who have friends struggling with mental health issues that the friend cannot address on their own. Believing that the app could be beneficial, they decide to download it for sharing with their friends.
- Help seeker (HS): Students whose mental health challenges have significantly disrupted their daily lives. While searching for professional help or resources, they encounter an application and download it.
- Curious explorer (CE): Students who notice the application's launch and promotion on campus. Motivated by mild curiosity or initial user incentives (e.g., rewards for early adopters), they decide to download and explore apps.
- Proactive maintainer (PM): Students without current mental health problems, but who have personally experienced or witnessed such issues previously. Concerned about their mental well-being, they download the app to access preventive resources and maintain good mental health.

Two hypothetical scenarios were presented sequentially to understand the students' attitudes and perceptions of the platform based on their motivations for use (see Table 1). The participants were briefly introduced to a demo version of a mental health app designed for college students and were asked to explore its features based on their assigned scenarios. They took on the role of the main character in each scenario and freely used the platform for approximately 10 min. During this usage, the app screen was recorded and the researcher stepped out of the lab. Once the participants completed their interaction with the app for each scenario, we conducted one-on-one semi-structured interviews. We asked participants about their usage of each key feature as well as the aspects they found satisfactory or unsatisfactory. Each interview lasted approximately 20 min. All interview questions used in this study are also included in the Appendix C.

3.4 Data Management and Analysis

All interviews were recorded with the participants' consent. Three researchers, including the first author, listened to the recordings

Table 1: Participant Demographics

ID	Gender	Age	Academic Status	Vignette
P1	Male	18	Undergraduate	Peer Supporter, Proactive Maintainer
P2	Female	19	Undergraduate	Help Seeker, Curious Explorer
P3	Female	30	Graduate	Help Seeker, Peer Supporter
P4	Female	31	Graduate	Curious Explorer, Help Seeker
P5	Male	22	Undergraduate	Help Seeker, Curious Explorer
P6	Female	23	Graduate	Proactive Maintainer, Peer Supporter
P7	Male	27	Graduate	Peer Supporter, Curious Explorer
P8	Female	28	Graduate	Help Seeker, Proactive Maintainer
P9	Male	22	Undergraduate	Curious Explorer, Peer Supporter
P10	Female	19	Undergraduate	Proactive Maintainer, Help Seeker
P11	Female	23	Undergraduate	Peer Supporter, Curious Explorer
P12	Female	29	Graduate	Proactive Maintainer, Help Seeker
P13	Female	28	Graduate	Curious Explorer, Proactive Maintainer
P14	Female	28	Graduate	Peer Supporter, Proactive Maintainer
P15	Female	20	Undergraduate	Proactive Maintainer, Help Seeker
P16	Male	21	Undergraduate	Curious Explorer, Peer Supporter

and refined the transcripts manually. All transcripts were uploaded and managed in secure cloud storage, accessible only to the research team. We employed an inductive thematic analysis approach based on Braun and Clarke’s methodology [6] to identify meaningful themes. Thematic analysis is a flexible qualitative method that enables researchers to identify response patterns and gain insights into participants’ perspectives. Through this process, the coders became familiar with the data while organizing individual codes into broader themes. For instance, “convenient and accessible counseling appointments (code)” → “reduced barriers to seeking help (subtheme)” → “online–offline service integration experiences (theme).” The three researchers iterated the coding process until a consensus was reached [17]. Consequently, three themes and ten subthemes were derived: (1) Platform-based online service experience, (2) Experience integrating online and offline services, and (3) Design requirements to improve platform usability.

As this study focused on observing participants’ natural interactions with the app rather than providing direct interventions, we just obtained informed consent for the collection of personal data, as well as the recording of interviews.

4 Findings

4.1 Platform-Based Online Service Experience

In this section, we examined participants’ experiences with online services (e.g., mental health news, self-assessments) and their feedback on usability and effectiveness.

4.1.1 From Trust in Content Providers to Trust in the App. Most participants highly valued the fact that this mental health management system was developed in collaboration with the campus counseling center, considering it to be a key factor in establishing trust. “When I hear that the app is developed by the healthcare center, I naturally trust it (P2, HS).” In particular, participants mentioned that they trusted the app because the health news content was written by professionals, including on-campus counselors or psychiatrists. For

instance, P3 commented on the health news feature, saying, “I really trust the content, because this app was developed by our school healthcare center, and it’s written by experts like counseling teachers (P3, HS).”

Some participants further highlighted that they could accept the content without skepticism because of this trust. P4 stated, “If the information is provided in Health News, I feel I can trust it. I don’t have to filter it out and can read it more comfortably (P4, CE).” It was also noted that references and the use of professional terminology within the content enhanced its credibility. This implies that clearly indicating the source is crucial for designing mental health content that fosters user trust. P12, for instance, said, “When I read the health information, I could see the references, so I trusted it enough (P12, PM).”

However, not everyone placed unwavering trust in the provided content. P9 (PS) adopted a more critical view, explaining that they do not consider the information to be “100% correct.” Despite this skepticism, they still found the health news feature potentially useful for supporting a friend: “I’m not sure if something like ‘ways to combat depression’ would really work, so I don’t take it as absolute truth. But if I have to help a friend, I might refer to it once and see if it helps (P9, PS).”

4.1.2 Broadening Content to Incorporate Self-Discovery and Everyday Topics. When asked what content topics they would like included on the platform, the participants frequently emphasized that the topics should not be overly serious. This sentiment was particularly common in scenarios such as the CE or PM, in which the participants did not necessarily experience mental health issues. For instance, P8 noted, “I’d like to see not only deep mental disorder items in the self-screening features but also something like a Myers-Briggs Type Indicator (MBTI) personality test—so even if someone doesn’t have a specific mental health issue, they can learn more about themselves (P8, PM).” Several participants, including P8, expressed interest in self-screening tools or health information addressing self-understanding, self-esteem, interpersonal relationships, and

learning styles. *"It'd be great to have a mild test like a learning style test. Undergraduates and graduate students all care about that (P8, PM)."*

4.1.3 Preferences for Linking Features. Participants responded favorably to the mental health content and counseling appointment prompts that appeared along with their self-screening results. Several participants noted that reading the recommended content helped them feel more at ease and provided practical ways to address their concerns. *"After I finished the self-screening, related health news content popped up. Thanks to this connection, I thought I could actually work on resolving my issues (P12, PM)."* Similarly, P9 felt that the linking between the features lowered the barriers to help-seeking, saying, *"After I took an anxiety test, I was provided with a counseling case study content. I learned how counseling works and how it can help me. I think that experiencing this link could change the minds of my friends who are hesitant about counseling (P9, HS)."*

4.1.4 Willingness to Recommend the Service to Others. Based on the aforementioned usage experiences, several participants indicated a willingness to recommend the platform to their friends. Some had peers struggling with mental health issues and thought that they might benefit significantly from the app. *"A lot of my friends at school are under mental stress, whether from relationships or academics, so this app could be really helpful for them (P7, PS)."* Others felt that they could confidently recommend the platform because it was run by the college, which made them feel trustworthy and that it was readily available. *"Because it's a college-run app, you can use it anytime. If a friend needs help, I'd probably say, 'Why don't you give the self-assessment a try (P4, CE)?"*

However, a few participants were hesitant to recommend it owing to concerns that they or their friends would be stigmatized because it was a mental health app. They worried that recommending the app might imply they "have a problem," leading to feelings of embarrassment or judgment. A quote from P13 illustrates this point. *"I could say, 'I think you might find this helpful,' but I worry they might take it as, 'I think something's wrong with you.' So, I'm cautious about recommending it directly (P13, CE)."* P10 explained that she would rather share their own experience as an indirect recommendation rather than directly suggesting that the friend needs it. *"There's a lot of shame and negative views when it comes to mental health issues. So instead of saying, 'I think you have a mental health problem, and you should try this app,' I'd probably say something like, 'Hey, the school's promoting this service, so I checked it out' (P10, HS)."*

4.2 Experience Integrating Online and Offline Services

In this section, we explore participants' experiences with the multi-domain app's features, including counseling appointments and registration of offline mental health programs (e.g., meditation sessions, group counseling programs).

4.2.1 Lowering Barriers to Help-Seeking. Several participants reported that the platform's overall bright and user-friendly atmosphere helped reduce the perceived hurdles of counseling, particularly during counseling appointments. For instance, P4 said, *"I might be reluctant to go for counseling in person because I don't feel like*

mental health problems are that critical. But with the app, my name and other details are automatically filled in, so it feels less burdensome, and anyone could easily give it a try (P4, CE)." Some participants compared this new appointment feature to the traditional process of signing up in person at a counseling center. Reflecting on a previous experience, P3 explained, *"Previously, going in person to the counseling center and filling out a form on paper felt like a burden in itself. But making a reservation on the app and typing out my worries in text helped me organize my thoughts ... that really lowered my fear about applying for counseling (P3, HS)."* A few participants also suggested adding brief information about counseling procedures or potential concerns (e.g., how confidential it is or whether it might impact their academic record) to reduce anxiety surrounding the first counseling session.

4.2.2 Self-Focused, Experiential Mental Health Programs. As noted in Section 4.1.2, the participants wanted the topics of offline mental health programs to be less heavy, similar to what they desired for health news and self-assessment content. P15 proposed offering casual, everyday activities such that students without urgent mental health issues could also participate freely. *"Since I don't currently have a mental illness, I'd prefer more relaxed, comfortable activities—like making a mood lamp—rather than really deep topics (P15, PM)."* Several participants, particularly those immersed in the HS vignette, suggested that programs aimed at self-understanding should be undertaken individually, instead of in groups, to lessen any potential discomfort. For instance, P3 said, *"I'd like a counseling program that keeps me from falling into feelings of self-deprecation, but I hope it's done one-on-one, rather than in a group (P3, HS)."*

4.2.3 Selective App Use Depending on Individual Situations. Several participants in the CE or PM scenarios said that they might not have used certain features presently. They explained that mental health management was not strictly necessary in their current situation. P5 said, *"From the CE perspective, I don't really need to apply for counseling, so I didn't try that feature (P5, CE)."* Nonetheless, P5 added that if they encountered mental health issues in the future, they would use the counseling appointment feature. P2 said, *"Even if I'm in a CE scenario rather than HS, just knowing these features are available whenever I might need them can be helpful (P2, CE),"* suggesting that exploring various features in advance could positively affect future help-seeking attitudes.

4.3 Design Requirements to Improve Platform Usability

In this section, we also investigated usability issues and technical limitations encountered while using the platform, identifying areas for improvement.

Although some participants found the conversational style of the mental health content in Health News easy to read—*"almost like an essay"*—others expressed discomfort with text-heavy content. For instance, P9 said, *"People these days are used to quick content like Shorts or Reels. So when there's a wall of text, it feels a bit off-putting (P9, CE)."* One participant noted that the presence of clinical terminology made it more difficult to follow and wished for simpler alternatives. Another usability concern was the categorization system for Health News. Several participants worried that the current

categorization might be ineffective. For instance, P5 said, “*There’s so much content under Health News that it feels unorganized. I might not even realize there could be user reviews there (P5, HS)*,” and emphasized that they prefer Health News to be strictly about health information, consistent with the name.

During the interviews, the participants also encountered functional glitches, such as certain pages failing to load or the back button not working properly. Because the app had not yet been fully developed or released, these issues were expected. We shared all the reported functional errors and user requests with developers and providers, and in September 2024, an updated version of the app was released to all students.

5 Discussion and Future Work

Overall, participants responded positively to the campus mental health management app, and some even expressed a willingness to share their experiences with friends once the app was officially released. We observed that this mobile platform has the potential to reduce barriers for college students seeking professional assistance. In particular, the app’s bright and inviting interface, as well as the counseling appointment feature integrated with users’ personal information, appeared to facilitate help-seeking. Participants also valued the platform’s multi-domain nature, which allowed them to access offline mental health services through an online interface, as well as the linking between features such as self-screening, health news, and counseling appointments. This nature can increase perceived susceptibility while reducing perceived barriers by enabling users to monitor their mental health status and making professional support more accessible when needed, ultimately motivating help-seeking behavior [34, 41].

Another interesting finding is that some participants preferred indirectly sharing the app rather than directly recommending it. This suggests that Korean students tend to favor approaches that reduce the psychological burden the recipient might feel when receiving mental health service recommendations. According to Stefancic et al. (2019), the shared experiences in peer-delivered mental health services contribute to building trust and fostering motivation, which can facilitate help-seeking behavior [37]. As a future direction, introducing an anonymous space for sharing reviews and recommendations of the app and its services could be considered to potentially enhance students’ motivation to seek mental health support. Moreover, participants indicated a preference for mental health services that minimized clinical terminology and focused on self-management—consistent with prior studies showing that college students favored digital mental health tools that do not directly emphasize mental health [25].

However, some users have shared concerns regarding the platform. Based on these comments, we derived additional design insights for mental health applications. Although text-based mental health content offers the advantage of privacy—readers do not need to worry about others seeing what they are viewing [25]—it may be helpful to incorporate multimedia elements (e.g., images or videos) to enhance readability. Therefore, clearer categorization strategies should be redefined to prevent user confusion.

In conclusion, our findings suggest that a platform that integrates existing campus mental health services and simplifies the process

of scheduling counseling has the potential to positively influence college students’ attitudes toward seeking help. Nevertheless, the relatively small sample size and recruitment of students already interested in the platform pose limitations that may have introduced biases. Additionally, the feedback collected reflects perceived user experience rather than actual user experience in a real-world setting, as it was based on participants’ responses to direct questions about inconveniences.

To implement digital mental health services effectively in campus settings, it is critical to design services that reduce the stigma surrounding help-seeking behaviors among college students. Such designs may also encourage individuals who have never pursued therapy to seek help [32]. In the next step, we need to conduct an in-depth examination of how students’ help-seeking attitudes, as well as stigma toward counseling and mental health, change when they freely use this platform in a campus setting using a larger sample of college students.

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A Screenshots of the Mental Health Platform Application

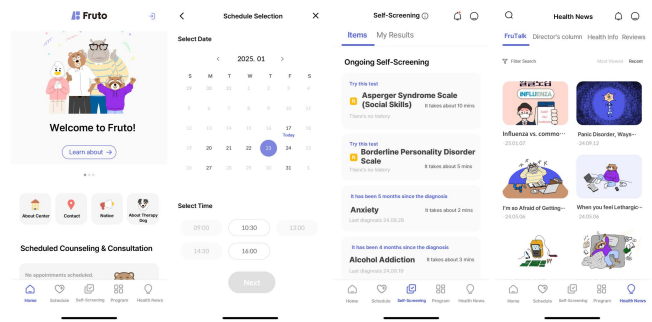


Figure 1: Example Screenshots from Fruto

B Vignettes Used In Interviews

Help Seeker During high school, you consistently ranked among the top students in your class, receiving frequent praise from teachers and friends alike. However, upon entering college, you experienced a significant setback when you failed to achieve the grades you had anticipated for the first time. Despite investing a similar amount of time and effort as your peers, the gap in academic performance weighed heavily on you. This sense of helplessness—feeling that hard work might not pay off—led to growing doubts about your career path and sense of purpose, intensifying your anxiety. In turn, these worries began to affect your daily life, creating a vicious cycle of poor concentration and further stress. While searching for a solution, you came across a campus mental health support app, and, in the hope that it might improve your circumstances, you decided to download it.

Peer Supporter Recently, your close friend A has seemed depressed and has been unresponsive to calls and messages. You finally managed to have a brief conversation with them, and learned that since choosing their major, A has been under intense stress and feeling powerless about their academics and future plans. You want to offer direct help, but each time you try, A compares themselves to you and other friends, revealing their vulnerability. Worried that A may need professional intervention, you hear about the release of a new mental health app on campus. You decide to download the app first, explore its various features, and then recommend the ones you believe could benefit your friend.

Curious Explorer You've heard news about a new mental health support app being released on campus. Although you don't have any particular mental health concerns, you are curious about new technology and on-campus services. By trying out the app in advance, you might be able to recommend it to younger students or friends when they're struggling. Additionally, you're interested in receiving rewards offered to early users. So, you decide to download the app and explore its various features.

Proactive Maintainer Although you haven't experienced any mental health issues yourself, you once witnessed someone close to you suffer severely from depression, requiring hospital visits and medication. This experience prompted you to research various mental health conditions—including depression—and become more proactive about monitoring your own mental well-being. While looking into different apps and services that offer mental health information, you came across a new mental health app provided by your school. As a preventive measure, you decide to download the app and give it a try.

C Semi-structured Interview Questions

For participants who have used any of the following features: Counseling Scheduling, Program Registration, Self-Screening, Health News, or About Center.

- How was your experience using this feature?
- Was the feature easy and convenient to use?
- Did the feature provide sufficient and clear information?

For participants who have not used any of the above features:

- What were the reasons for not using the feature(s)?
- Were you aware of these features prior to this discussion?
- Do you think you might use these features in the future? If yes, under what circumstances would you consider using them?
- What additional information or improvements would you encourage you to use these features?

Regarding the app as a whole:

- What are your overall impressions of using the app?
- Which feature(s) did you find most useful? And why?
- Are there any areas where you feel the app could be improved?
- Would you recommend this app to a friend? And why?